



Internal Medicine Follow■Up Note – Lymphadenopathy Review and Next Steps

Service: Internal Medicine (General) – Follow■Up

Location: NorthStar Medical Clinic – Waterfront

Date of Service: 2025-03-24

Reason for Visit: Review imaging/labs; chronic lymphadenopathy; hematuria surveillance

Provider: Staff Physician (template note, synthetic)

****Visit Type:**** In■person follow■up for chronic lymphadenopathy and review of interval investigations.

****Subjective:**** The patient reports gradual enlargement of lymph nodes over approximately 4–5 years. Prior biopsies and blood work have been nondiagnostic. He denies systemic “B symptoms” and reports generally feeling well aside from transient jet lag after recent travel. He seeks diagnostic certainty and wishes to understand whether further testing (repeat biopsy, nuclear medicine imaging) is warranted. He also reports a history of intermittent microscopic hematuria and wishes to review urinary and renal imaging results.

****Objective / Exam:**** Well appearing, afebrile, vitals within normal limits. Neck: notable enlargement of upper anterior cervical nodes; non■tender; not rock hard; no overlying skin changes. Cardiopulmonary exam unremarkable by clinic template. Abdomen non■distended; no clear hepatosplenomegaly appreciated.

****Results Reviewed – Urine Studies (2025■03■24):**** Urinalysis within normal limits; urine culture negative; urine cytology negative; albumin■creatinine ratio within normal limits. CRP 9.1 mg/L (elevated).

****Results Reviewed – Ultrasound (2025■03■24):**** Multi■site ultrasound of neck, bilateral groin, abdomen/pelvis, and bilateral axilla demonstrates well■defined hypoechoic masses in the anterior neck with ****thick cortical mantles****. Dominant right level 2A node measures ****37.8 × 11.5 × 27.5 mm**** (larger than prior). Additional right level 2A node ****23.3 × 5.8 × 19.9 mm**** (new). Left level 2A node ****22.3 × 9.4 × 19.6 mm**** (larger) and left level 2B node ****19.9 × 7.8 × 16.2 mm**** (larger). Groin: well■defined nodes medial to neurovascular bundle up to ****24 × 7.5 × 13.8 mm**** on the right and ****15.3 × 4.2 × 12.8 mm**** on the left, described with thin cortical mantles. Axillae: subcutaneous nodes with thin cortical mantles; dominant right axillary nodes ****21.5 × 7.7 × 10.8 mm**** and ****18.8 × 5.8 × 7.9 mm****; dominant left axillary nodes ****25.5 × 8.4 × 10.4 mm**** and ****16.4 × 5.6 × 8.3 mm****. Renal ultrasound normal bilaterally; prostate volume 13.2 cc; post■void residual 63 cc.

****Assessment:**** Persistent, enlarging, asymptomatic multi■station lymphadenopathy of unclear etiology with intermittently rising CRP. Recurrent microscopic hematuria under surveillance with reassuring workup. Incidental ileitis on prior colonoscopy without GI symptoms. Small pulmonary nodules previously assessed as low risk.

****Plan:**** (1) Second opinion Internal Medicine follow■up arranged (IM referral already placed; follow■up planned mid■May). (2) Refer back to ENT for consideration of repeat biopsy given interval growth and the acknowledged possibility of sampling limitation in prior FNA. (3) Repeat CRP, CBC, peripheral smear; consider LDH/SPEP if clinically indicated. (4) Infectious Diseases referral to review travel history and evaluate for subclinical infection where appropriate. (5) Discussed gallium scan as an option; patient prefers to avoid additional radiation for now given current well■being. (6) Continue annual renal/bladder ultrasound surveillance for hematuria; next due ~2026■03. (7) Patient provided consent to use de■identified data for AI■assisted differential exploration for rare conditions (research/decision support only).



Addendum / Comparison and Administrative Notes

Comparison statements reflect the most recent prior studies available in the network at the time of reporting. If external imaging exists, formal comparison may be limited until images are imported. Clinicians are encouraged to interpret findings in the context of the patient's broader longitudinal record and to correlate with physical examination and symptoms.

Quality Assurance: This report was auto-formatted from a structured radiology/clinic template and reviewed for internal consistency. Minor variations in measurements may occur due to technique, probe position, and interpreter variability.